6.6 Gastrectomy

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Gives his abdominal pain 7/10 **Objective** • Incision in the upper abdomen • NG tube in situ	**ACTUAL** **Acute pain** related to surgical incision as evidenced by pain score 7/10.	Pain 3/10 or less within 24 hours.	• Give analgesia regularly as prescribed. • Never reposition the NG tube; report it if it comes out of place. • Support the incision when he coughs. • Get him moving early.	Pain controlled: **goal met**.
Subjective • "I get dizzy and sweaty after eating." **Objective** • Symptoms come 15–30 minutes after eating	**ACTUAL** **Imbalanced nutrition: less than body requirements** related to reduced gastric capacity and dumping syndrome as evidenced by symptoms after meals.	Eats without dumping symptoms.	• Advise small meals often, high in protein and low in simple sugar. • Drinks go between meals, not with them. • Lie down for 20–30 minutes after eating. • Teach that he will need vitamin B12 replacement long term, as prescribed.	No dumping symptoms: **goal met**.
Subjective • *No specific complaint* **Objective** • Major abdominal surgery • Drain in situ	**RISK** **Risk for infection** as evidenced by risk factors: surgical wound and anastomosis.	No infection, no anastomotic leak.	• Watch temperature, the wound and the drain. • Report fever, a fast heart rate or pain that is getting worse. • Aseptic technique for wound care.	No infection: **goal met**.